

Sample lecture: Odontogenic oral pathology

Odontogenic cysts are epithelial-lined cavities derived from remnants of the tooth-forming apparatus and represent a core topic in oral pathology education.

Radicular cysts typically arise from inflammatory stimulation at the apex of a non-vital tooth and are among the most frequently encountered jaw cysts in clinical practice.

Dentigerous cysts envelop the crown of an unerupted tooth and are considered developmental lesions associated with impacted teeth rather than purely inflammatory processes.

The odontogenic keratocyst may behave aggressively with higher recurrence risk compared with many other cystic lesions, prompting vigilant radiographic follow-up after surgical treatment.

Ameloblastoma is the most common benign odontogenic epithelial tumor and often presents as a slowly expansile radiolucency favoring the posterior mandible on panoramic imaging.

Clinical correlation among patient age, tooth vitality testing, radiographic borders, and eventual histopathology remains essential when distinguishing inflammatory cysts from keratocystic odontogenic lesions.

Myxoma and odontogenic fibroma illustrate mesenchymal odontogenic tumors that require careful imaging assessment because local infiltration can challenge conservative surgical planning.

Students preparing for boards should practice articulating why inflammatory radicular disease differs from developmental dentigerous pathology despite overlapping jaw radiolucencies.